

Patient ID: S (Chair)Period: May 29 to June 28, 2024Report Date: June 29, 2024Coverage: 584/736h (79%)

Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

Elevated Breathing > 24 brpm

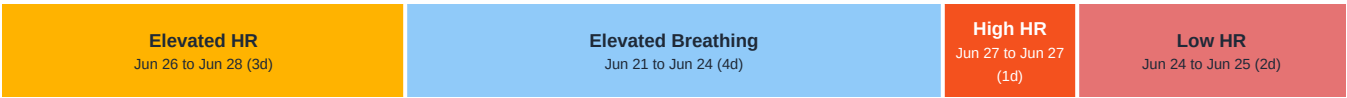
Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm

Patient clinical status over 31 days from May 29 to Jun 28



Last 24 Hours

Last 24 Hours: ELEVATED

30-Day Tier: RED

Vital	Avg	Min	Max
Heart Rate	91 bpm	60 bpm	109 bpm
Breathing	12 brpm	9 brpm	25 brpm

Last 24 hours: two elevated heart rate episodes overnight; one high heart rate episode overnight.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jun 28	12 AM to 3 AM	8h	3	Elevated Breathing	12	9	25	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jun 28	3 AM to 6 AM	5h	2	Elevated Heart Rate (brief)	97	83	104	Check temp, hydration, pain; review for arrhythmia or infection
Jun 28	12 AM to 3 AM	3h	1	High Heart Rate (brief)	102	91	109	Check temp, hydration, pain; review for arrhythmia or infection

15 episodic events Detected, spanning 39h total. Conditions: Elevated Breathing, Elevated Heart Rate, High Heart Rate, Low Heart Rate. Priority grade: High

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
15	39h	Elevated Heart Rate	<1	79%

Major Findings: Sustained elevated HR (avg 97, peak 109)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Jun 27	8 PM to 3 AM	7h	1	High Heart Rate	102	91	109	Check temp, hydration, pain; review for arrhythmia or infection
Jun 24	2 AM to 3 AM	4h	2	Low Heart Rate	43	40	46	Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
Jun 21	5 PM to 6 PM	13h	4	Elevated Breathing	28	15	44	Check SpO2 and lung sounds; assess for fluid overload or early infection
Jun 26	10 AM to 11 AM	15h	8	Elevated Heart Rate	97	60	106	Check temp, hydration, pain; review for arrhythmia or infection

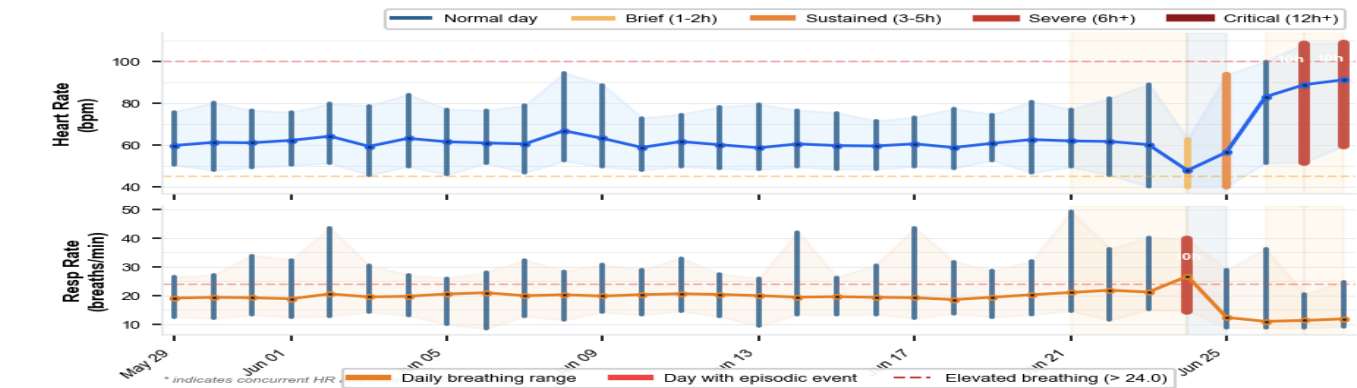
The P5 to P95 heart rate spread was 40 bpm (53 to 93), indicating significant cardiac variability.

Suggested Clinical Review Actions

- Elevated Heart Rate (avg 97 bpm, peak 106 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Elevated Breathing (avg 28 brpm, peak 44 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- High Heart Rate (avg 102 bpm, peak 109 bpm): Check temp, hydration, pain; review for arrhythmia or infection
- Low Heart Rate (avg 43 bpm, min 40 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic
- Overall trajectory shows 4 unstable phases with 40 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

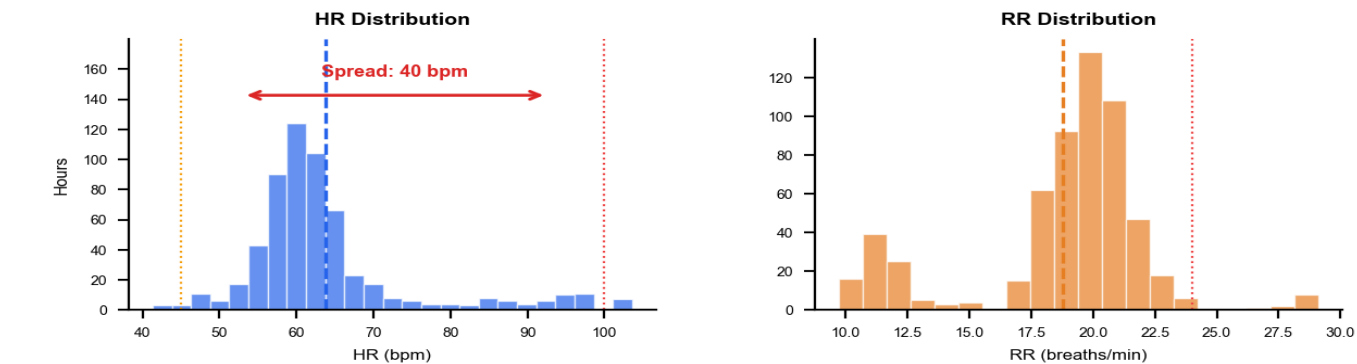
Clinical Pattern Observations:

- 1. Sustained finding: 10h continuous Elevated Breathing on Jun 24.
- 2. High variability: HR spread 40 bpm (52 to 92).
- 3. Breathing variability: RR spread 11 brpm (11 to 22).

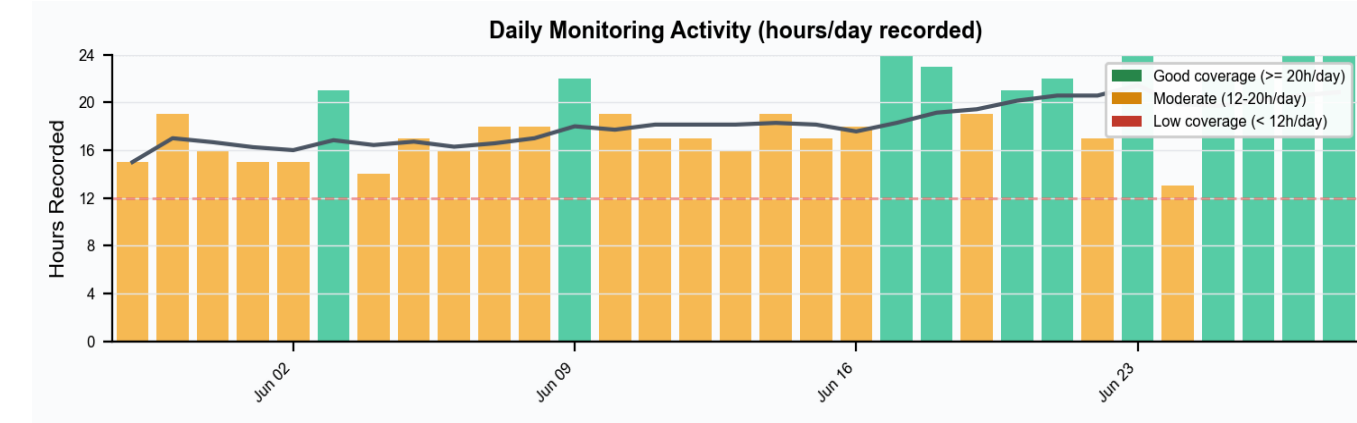


Red bars indicate days with detected episodic events.

Vital Sign Distribution



Monitoring Activity



Bars indicate monitoring hours/day. Red threshold line indicates clinical monitoring baseline.

■ HR episode ■ RR episode ■ Recorded, no episode ■ No data

Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.